

Mohr Insurance — Dynamic Sales Script

Script version 5 · branched reference & agent worksheet · four tracks, each with its own questions · umbrella priced from the needs assessment

Standing compliance rule

BEFORE ANYTHING ELSE

A client who is 65 or older in California cannot be sold cancer / heart attack / stroke cover or skilled nursing cover. Because neither can be sold, **do not embed either exposure** — skip the major-exposures set-up, the CHS conversation and the skilled nursing conversation entirely.

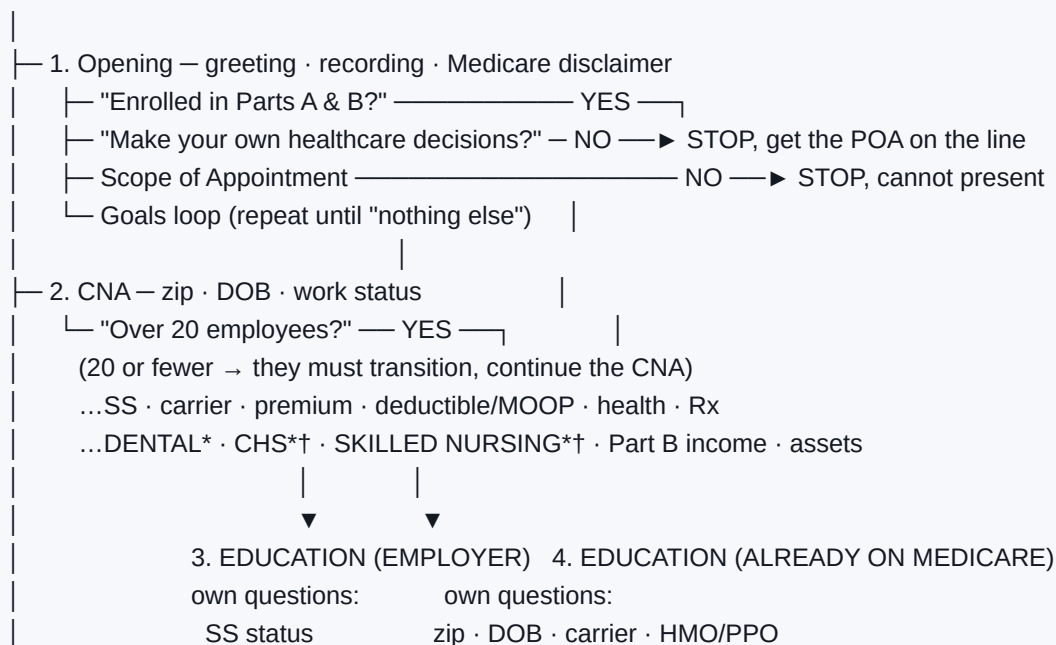
Dental can still be sold. Ask whether it is important to them as normal; it still prices \$50 into the Medicare Supplement umbrella. That leaves the Advantage umbrella at \$0, since an Advantage plan already bundles dental.

Gold / Silver / Bronze is cancer, heart attack and stroke cover, so it cannot be sold either. On the employer track there is no Medicare plan to offer in its place, so that call ends after the delay recommendation and the CMS L564 education.

Their zip code and date of birth decide all of this, and both are captured early on every track.

How this script branches

EVERY CALL



carrier · HMO/PPO premium · max OOP · hit it
 premium · max OOP DENTAL* · CHS* · SKILLED NURSING*
 hit it → commitment ↓
 ↓ "Want the Supp vs MA rundown?"
 Rec 1: delay Medicare | — YES —► education → pricing
 (branches on SS) | — NO —► straight to pricing
 Rec 2: Gold/Silver/Bronze ↓
 ↓ 3 STEP CLOSE
 close (umbrella · lock in plan · effective date)

5. MEDICARE EDUCATION — T65 only

Scenario 1 on SS (auto-enrolled) / Scenario 2 not on SS (manual)

- Original Medicare → 20% exposure → paper check
- 6. Supplement → 7. Advantage → 8. Pricing with umbrella
- leaning → formal recommendation
- Scenario A (not on SS) / Scenario B (on SS)
- underwriting urgency → address → objections + pre-emptive close

* umbrella drivers (T65 track) † skipped entirely when the client is 65+ in California
 Gold / Silver / Bronze is the product on the employer track and on both already-on-Medicare
 branches. The \$50/\$100/\$150 umbrella built from the needs assessment is T65 only.

Track	How you land on it	Its own questions	Close
Employer coverage	Not on Medicare · still working · over 20 employees . Exits at the work-status question.	SS · carrier · HMO/PPO · premium · max OOP · hit it	Gold / Silver / Bronze, then address
Already on Medicare	Parts A & B active. Exits right after the goals loop, then splits again by what they hold.	4A Advantage: HMO/PPO · premium · CHS · SEP check 4B Supplement: premium only, then the standardization pitch	4A: 2 or 3 steps by timing 4B: 3 steps
T65 — not on SS	Not on Medicare, not drawing Social Security	The full needs assessment in section 2	Scenario A
T65 — on SS	Not on Medicare, drawing Social Security	The full needs assessment in section 2	Scenario B

*An employer plan with **20 or fewer employees** makes Medicare the primary payer — the script says outright that those clients **must transition to Medicare**, so they stay in the needs assessment and run the T65 track.*

Umbrella pricing — T65 track only

*Clients already on Medicare and clients on employer coverage are sold **Gold / Silver / Bronze** instead. This table does not apply to them.*

What they told you in the CNA	Medicare Supplement	Medicare Advantage
Cancer / heart attack / stroke is a concern	+\$50	+\$50
Skilled nursing is a concern	+\$50	+\$100
Dental is important to them	+\$50	included with the plan
Maximum umbrella	\$150	\$150
California, 65 or older — cancer and skilled nursing removed, dental only	\$0 or \$50	\$0

Quote	Formula
Medicare Supplement	Plan G premium + Part D $\approx$ \$10 + umbrella (\$50 / \$100 / \$150) = one monthly price
Medicare Advantage	MAPD premium (usually \$0) + umbrella (\$50 / \$100 / \$150) = one monthly price

Quote the single all-in number. Do not itemize it out loud unless they ask.

65 or older in California: only the dental component survives. The Supplement umbrella is \$50 if dental matters to them and \$0 if it does not; the Advantage umbrella is \$0 either way. When the umbrella lands at \$0 there is nothing to sell today, and the close drops to two steps.

1 • Opening

Greeting and webinar confirmation

"Hey [Name], this is [Agent name] with Mohr Insurance Services calling for our scheduled Medicare appointment... how are you today?"

Wait for their response.

"Thank you for booking the call today. Hopefully I'll be able to help you out and answer any questions."

Call recording disclosure

"Now [Name], before we continue, I have to read these compliance disclosures before we dive in. You know we take compliance very seriously, so I need to let you know that **this call is being recorded for quality and compliance purposes.**"

Required Medicare disclaimer

"We do not offer every plan available in your area. Any information I provide is limited to the plans we represent. You can always visit Medicare.gov or call 1-800-MEDICARE to review all available options."

"Now, almost done here!"

Required verbatim. Do not paraphrase or skip.

Eligibility check branch point

"Before we go further, are you currently enrolled in Medicare Parts A and B?"

IF YES

They are **Avatar 4 — Currently on Medicare**. They will get the current plan review instead of an enrollment paragraph, and Scenario C at the close.

"And do you personally make your own healthcare decisions?"

IF NO

Stop. You cannot present or enroll without the authorized representative or power of attorney on the line. Ask who handles their healthcare decisions and get that person on the call, or reschedule with them present.

Scope of Appointment permission

"Now for today's call, we may discuss: Medicare Advantage Plans · Medicare Supplement Plans · Prescription Drug Plans · Ancillary Products. Do I have your permission to discuss these options with you today?"

*Wait for a **clear verbal YES**.*

IF NO

Do not proceed. Without a Scope of Appointment on the recording you cannot discuss any of the four product categories.

Transition into the CNA — the goals loop

"Perfect. I like to start with what's called a Client Needs Assessment. This helps me gather the right information and make sure I'm steering you in the best direction. But before we get started, I always like to ask: what would you like to accomplish today?"

Write down what they say — their words.

1.

2.

3.

"Perfect, so what I'm hearing is (repeat their goal). Does that sound about right?"

"Perfect, is there anything else you want to go over today?"

***Loop** until they have nothing else. You read every one of these back in the formal recommendation.*

PARTS A & B ALREADY ACTIVE

Skip the needs assessment below entirely and go to **section 4, Education (Already on Medicare)** — that section has its own questions.

2 · Client Needs Assessment

Zip code

"So for starters, what's your zip code?"

Repeat the zip code back. → "Awesome, got that down."

Zip:

Date of birth

"Now what is your date of birth?" → "Perfect, got that here."

DOB (mm/dd/yyyy): Age: Birth month:

Work status branch point

"Tell me a little bit about your current situation — are you still working, are you retired, what does that look like?"

IF STILL WORKING

"Does your employer have over 20 employees?"

Over 20 → **stop the needs assessment here** and go straight to section 3, Education (Employer). That section asks its own questions.

20 or fewer → they must transition to Medicare. Continue through the rest of the needs assessment below.

Work status:

Social Security status branch point

"Are you drawing Social Security yet?"

YES

Automatically enrolled into A & B · Part B deducted from their check · **Scenario B** at close.

NO

We enroll them manually · billed quarterly for the first quarter · **Scenario A** at close.

Current insurance carrier and source branch point

"Tell me a little bit about your current health insurance — is that through your employer or through the marketplace?"

"What carrier is that with?" · "And is that an HMO... PPO?"

Source: Carrier: HMO / PPO:

Monthly premium

"What are you paying for that right now on a month to month basis?"

\$ /mo

Deductible and max out of pocket

"And then you may not know this next part off the top of your head, but if you have your insurance card it should read off of there. What is your deductible and maximum out of pocket?"

Deductible: \$ Max out of pocket: \$

The max out of pocket is the entire basis of the employer-track pitch. Do not leave this blank.

"Have you hit that deductible or max out of pocket, and if so what happened?"

Health history

"And that leads me into my next question — sounds like you're pretty healthy / *have a little bit of a health history*. Is there anything with your recent health we should know about in the past 5 years?"

Pick the half of that sentence that matches what you've heard so far. Underwriting matters here.

"Okay, and are you taking any prescriptions right now, and what are they actively treating?"

Dental importance

umbrella driver

still asked in CA at 65+

"My next question would be, is dental insurance important to you, and do you currently have that with your employer or through the marketplace?"

IF YES

+\$50 to the Medicare Supplement umbrella. No effect on the Advantage umbrella — an Advantage plan already bundles dental.

Major exposures — the set-up

skipped in CA at 65+

"Now, in that webinar do you remember how we highlight major exposures?"

"Major exposures can really stem from any plan — whether it's your current plan with [carrier], or a Medicare Supplement or Advantage plan — but really in relation to two specific things."

Cancer, heart attack, stroke

umbrella driver

skipped in CA at 65+

"For starters — cancer, heart attack, stroke. Do we have any personal or family history with the big three?"

"I really appreciate you sharing that with me... and the reason we bring this up — I'll share a personal note. My grandmother had cancer three times, and she was on Medicare and had great coverage, but even with that coverage she had to pay **\$15,000** out of her own pocket for non-approved expenses. **Is something like that going to be a concern for you?**"

IF YES

+\$50 to both umbrellas.

Skilled nursing

umbrella driver

skipped in CA at 65+

"Now the next big exposure we see is skilled nursing. Do you have any personal or family history with that, and are you familiar with how that coverage functions with Medicare?"

"Medicare will help cover up to day 100 as long as you had a **3 day hospital stay** and are **showing signs of improvement**... but after day 100 you are responsible for all costs. And it's really important we focus on that 3

day hospital stay and showing signs of improvement."

"I had a client who was in skilled nursing for ten days and her insurance plan, Kaiser, said she was not improving and doing the exercises because she had a stroke — so they ended her care and she had to go home and pay **\$20,000** out of pocket for home care. So I'm assuming this will be a concern as well?"

IF YES

+\$50 to the Supplement umbrella · +\$100 to the Advantage umbrella (the Advantage side is buying skilled nursing coverage).

"And yeah, we'll come back to that in greater detail, but I really just wanted to make sure you're aware of these exposures."

Part B premium — income qualification

"Now, when you transition to Medicare you'll be responsible for a Part B premium — however that premium is going to be very dependent on your household income. Do you know where that's at at this moment in time?"

"And has that been pretty consistent over the last two years?" ... "And the reason I ask is they always look back two years prior."

Income: Consistent 2 yrs:

Retirement assets

"And another thing that can impact your Part B premium, which not a lot of people think about — do you have any retirement accounts that have over \$200,000 in qualified assets or more?"

"And the reason I ask is because when we plan to supplement our income with the money within those accounts, that can impact your Part B premium. Have you had the chance to talk to a retirement specialist yet?"

IF NO

"Okay, well we have a full partnership on hand, so if you ever want to sit down and have that conversation it is a service we offer **free of charge**, and we can look at getting you scheduled for that once we're done here — okay?"

Close out the CNA

"Awesome, that wraps up the needs assessment. Now we're going to walk through your options and answer the questions you had at the beginning."

3 · Education (Employer)

*Employer coverage with over 20 employees. This section has **its own questions** — the client left the needs assessment at the work-status question, so nothing below has been asked yet. This track ends here: no Supplement, no Advantage, no umbrella.*

Social Security status branch point

"Are you drawing Social Security yet?"

*If **yes**, take note — they will be automatically enrolled in Part A and must **request to delay Part B**. This changes two lines further down.*

Carrier and plan

"Tell me a little bit about your current health insurance — is that through your employer?"

"What carrier is that with?" · "And is that an HMO... PPO?"

Carrier: HMO / PPO:

Monthly premium

"What are you paying for that right now on a month to month basis?"

\$ /mo

Max out of pocket

"And then you may not know this next part off the top of your head, but if you have your insurance card it should read off of there. What is your maximum out of pocket?"

\$

"Have you hit that max out of pocket, and if so what happened?"

Get their commitment before you move on. This is the only exposure conversation on this track.

"Yeah, that makes sense. So I have all the information I need and have two recommendations for you."

Recommendation #1 — delay Medicare branches on SS

NOT DRAWING SOCIAL SECURITY

"So right now this transition to Medicare does not make much sense for you, so my first recommendation would be to **delay Part A and Part B entirely**. You have a strong plan with your employer and your premiums are pretty low."

DRAWING SOCIAL SECURITY

"So right now this transition to Medicare does not make much sense for you, so my first recommendation would be to **delay Part B entirely** — but you will be automatically enrolled in Part A, because you're drawing Social Security. You have a strong plan with your employer and your premiums are pretty low."

Peace of mind on penalties

"And I want to make sure you are at peace of mind with penalties, since that's always a big concern. **You won't be penalized for delaying Medicare** since you have creditable coverage with your employer. Are we clear on that?"

Wait for the response. Do not move on without a yes.

CMS L564 — the hall pass branches on SS

"Now when you do decide to retire, or you lose that creditable coverage, that's when you take action. That's when you take **CMS L564** — it's a form you'll take to your HR department and they'll help you fill it out. Then you'll take that form and you're gonna submit it along with your application for **[not on SS: Part A and Part B / on SS: Part B]**. That form is basically your **hall pass that voids all penalties**. Are we clear on that?"

CALIFORNIA, 65 OR OLDER — STOP HERE

Gold / Silver / Bronze is cancer, heart attack and stroke cover, so it cannot be sold, and there is no Medicare plan to offer in its place because they are staying on employer coverage. **End the call as education**. Answer their questions, make sure they know to call when they lose creditable coverage, and note when to follow up.

Recommendation #2 — cover the gaps

"Now my second recommendation would be to protect you from the gaps that employer plans typically leave behind."

STEP 1 — REITERATE THEIR EXPOSURE

"So right now, your employer plan has a max out of pocket of **[max OOP]**."

STEP 2 — EXPLAIN THE RISK

"What this means is, god forbid you have a bad health year, it's very likely you will pay **[max OOP]** for your healthcare **plus** potentially have loss of income — which are two significant exposures."

STEP 3 — PRESENT GOLD / SILVER / BRONZE

"That's where our **Gold, Silver and Bronze** options come in — they're designed to cover those big out-of-pocket risks while you stay on your employer plan."

"Now [first name], I want you to write down Gold, Silver and Bronze, and let me know once you've got that down."

*Wait for confirmation before **each** tier below. One at a time — this pace is the whole close.*

Tier	Cancer	Heart attack & stroke	Total benefit	Monthly
Gold	\$30,000	\$30,000	\$60,000	\$150
Silver	\$20,000	\$20,000	\$40,000	\$100
Bronze	\$10,000	\$10,000	\$20,000	\$50

"Awesome, so based on these prices, which option do you feel like you can comfortably afford on top of your employer coverage?"

Ask, then stop talking.

"Sounds good, so what would be a good address to send policy packets and information?"

*Handle objections and **always end with a pre-emptive close.***

4 • Education (Already on Medicare)

Parts A & B are already active. This section is **self-contained** — it has its own questions and its own closes, and it does not run sections 5 to 8. Those are for T65 clients only.

4.0 Shared opening — run this before you branch

"So for starters, what's your zip code?"

Repeat the zip code back. → "Awesome, got that down."

"Now what is your date of birth?" → "Perfect, got that here."

Zip: State: DOB (mm/dd/yyyy): Age:

"Tell me a little bit about your current health insurance — what carrier is that with?"

Branch point decides the whole call

"And is that a **Medicare Advantage** or **Medicare Supplement** plan?"

MEDICARE ADVANTAGE

→ **Branch 4A**

MEDICARE SUPPLEMENT

→ **Branch 4B**

THEY DON'T KNOW

"Does your card say the carrier's name at the top with copays listed on it, or does it say Medicare Supplement Plan G or Plan N?"

Copays for doctor visits → **Advantage**. A plan letter on the card → **Supplement**.

CALIFORNIA CHECK — DO THIS NOW

Off the zip code and date of birth you just took. California **and** 65 or older means no ancillary: no umbrella, no Gold / Silver / Bronze. Use the California variant close inside whichever branch you land in. Everything else in the branch runs the same.

The exposure conversation is skipped too — there is no critical illness product to sell here, so it is not embedded.

Branch 4A — Medicare Advantage

4A.1 CNA continuation

"And is that an HMO or a PPO?"

"What are you paying for that right now on a month to month basis?"

HMO / PPO: Premium: \$

No max out of pocket on this branch — the exposure is carried entirely by the critical illness coinsurance percentage in 4A.4.

MAJOR EXPOSURES — CANCER, HEART ATTACK, STROKE SKIPPED IN CA AT 65+

"Gotcha, now in that webinar do you remember how we highlight major exposures? Major exposures can really stem from any plan — whether it's your current plan with [carrier], or any other Medicare plan — but really in relation to **three specific things**."

"Which are cancer, heart attack, stroke. Do we have any personal or family history with the big three?"

"I really appreciate you sharing that with me... and the reason we bring this up — I'll share a personal note. My grandmother had cancer three times, and she was on Medicare and had great coverage, but even with that coverage she had to pay **\$10,000** out of her own pocket for non-approved expenses. Is something like that going to be a concern for you?"

Get their commitment.

"And yeah, we'll come back to that in greater detail, but I really just wanted to make sure you're aware of that exposure."

4A.2 SEP check branch point

"Now before I make any recommendations, quick timing question. Has anything changed recently? Did you move, lose other coverage, or start getting any help paying for prescriptions?"

"And has [carrier] mailed you anything about your plan changing?"

Answers	Route
All no, and today is outside Oct 15 – Dec 7	Outside AEP
Any yes, or today is inside Oct 15 – Dec 7	In AEP / SEP

If they said yes: "That may open a window for you. Let me verify it before I promise anything."

Never tell them they have a special enrollment period until you have confirmed it.

4A.3 Timing frame — pick one

OUTSIDE AEP

"So here's where we're at. You're locked into [carrier] until October 15th. I can't change your medical plan today, and I'm not going to pretend otherwise."

"What I can do is show you where this plan leaves you exposed, and show you how we can get rid of that exposure. Sound fair?"

IN AEP / VERIFIED SEP

"Good timing. We're in the window where I can actually change your plan. We'll do it in the right order — exposure first, then the plan itself. Sound fair?"

4A.4 Exposure

"Grab your insurance card for me. I want the exact plan name off the front — [carrier] has a few plans in your area and I want to be sure I'm looking at yours."

Pull up the Summary of Benefits for that exact plan and zip — you need their critical illness coinsurance percentage. It is the only number in this section and the line does not land without it.

Exact plan name: Critical illness coinsurance: %

STEP 1 — EXPLAIN THE RISK

"So I found your current plan with [carrier] and I am looking through the summary of benefits, which is just the list of items that your plan covers. I can see here that your biggest exposure is to **critical illnesses**. Your plan has you paying [X]% of all treatment costs, god forbid you get diagnosed with a critical illness. **Were you aware of that?**"

STEP 2 — ADDRESS THE CONCERN

"And is that going to be a concern for you?"

Get their commitment.

STEP 3 — PRESENT CHS

"That's where our Gold, Silver and Bronze options come in. They're designed to cover those big out of pocket risks while you stay on your plan with [carrier] — and if you decide to transition to a new plan later on."

CALIFORNIA, 65 OR OLDER

Skip 4A.5 and go to **4A.7**.

4A.5 Present Gold / Silver / Bronze

"Now [first name], I want you to write down Gold, Silver and Bronze, and let me know once you've got that down."

*Wait for confirmation before **each** tier.*

Tier	Cancer	Heart attack & stroke	Total benefit	Monthly
Gold	\$30,000	\$30,000	\$60,000	\$150
Silver	\$20,000	\$20,000	\$40,000	\$100
Bronze	\$10,000	\$10,000	\$20,000	\$50

"Awesome, so based on these prices, which option do you feel like you can comfortably afford on top of your current coverage?"

4A.6 The close — pick one

2 STEP CLOSE — OUTSIDE AEP

"Based on everything you told me today, [read back your notes], my recommendation is the [Gold/Silver/Bronze] umbrella at \$[X] a month, and a full review of your [carrier] plan in October."

PAUSE ... the first one to talk loses. Then: "Still have that paper? Write this down. Two steps."

1. We get your umbrella coverage set up for \$[X] a month, today.
2. We put a date on the calendar in October to review your [carrier] plan for next year.

"Step 1 is the only thing we knock out today. The reason we handle this now instead of waiting until October is because we need to make sure we can get you approved through underwriting... and once you're approved, this locks in your rate based on your age right now."

Book the October appointment on the calendar before the call ends. "Call me in October" is not an appointment.

3 STEP CLOSE — IN AEP OR VERIFIED SEP

"Based on everything you told me today, [read back your notes], my recommendation is the [Gold/Silver/Bronze] umbrella at \$[X] a month, plus a full review of your [carrier] plan in the next 48 hours."

1. We get your umbrella coverage set up for \$[X] a month.
2. We get back together within 48 hours and do a full review of your [carrier] plan.
3. Your new coverage goes into effect on **[effective date]**.

"Step 2 is its own appointment because I have to run your doctors and your medications against every plan in your zip code, and that takes a good amount of time. So we'll get that booked within 48 hours of today... okay?"

Book the follow-up live, inside 48 hours. Get the full doctor list, prescriptions with dosages, and preferred pharmacy before you hang up, or you have nothing to run.

"I'll handle the entire enrollment process for you... and what's a good address to send policy packets and information to?"

*Handle objections and **always end with a pre-emptive close.***

4A.7 California variant — 65 or older, no ancillary

Run 4A.1 through 4A.4 **without the exposure conversation** — there is no critical illness product to sell, so it is not embedded. Do not present Gold, Silver or Bronze.

OUTSIDE AEP

"So here's my honest recommendation. Your plan is locked until October 15th, so there's nothing for you to buy today and I'm not going to invent something. What I want is a spot on the calendar in October."

"In September [carrier] mails you a book called the **Annual Notice of Change**. It tells you what's changing about your copays and your drug list for next year. Almost nobody reads it. I read it for you, and I compare your plan against everything else in your zip code."

1. We get your review scheduled for the week of **[October date]**.
2. Any changes we make go into effect January 1st.

"Before we hang up, let me confirm your doctors, your prescriptions, and a good street address so I'm ready to go in October."

Book the appointment live. Confirm doctors, prescriptions and pharmacy in the CRM — that is the deliverable on this call.

IN AEP

"Good timing. We're in the window where I can actually make a change for you, so let's go ahead and review your plan."

Go directly into plan comparison and enrollment.

Branch 4B — Medicare Supplement

4B.1 CNA continuation

"What are you paying for that right now on a month to month basis?"

Premium: \$ Plan letter:

4B.2 Education — federal standardization

"And [name], are you familiar with Medicare Supplements being federally regulated?"

"So Medicare Supplements are federally regulated. That means a Plan G with [their carrier] and a Plan G with any other company in the country cover the **exact same things**, because the government requires them to. So no matter which insurance carrier you go with, your coverage will be the same."

"Does that make sense so far?"

"So if the coverage is identical by law, then the only thing that's actually different between one company and the next is **the price**, and how often that company raises rates on their people. That's it."

"Which means when I go shop this for you, I'm not asking you to give up a single thing. Same coverage, same doctors, same benefits — different price. And looking at it right now, we could get you the same coverage with **[appointed company]** for **[price]** per month. How does that sound?"

Quoted carrier: Their price: \$ Monthly saving: \$

UNDERWRITING — FRAMED HONESTLY, FRAMED TO MOVE

"Now here's the one catch. Because you're past your original open enrollment window, moving to a lower priced plan goes through **underwriting**, which is just a handful of health questions."

"So here's what I always recommend — we put the application in and we see what happens. If you're approved, you're paying less every single month for the exact same coverage, and we cancel the old policy. If you're not approved, absolutely nothing happens. Your coverage with [carrier] stays exactly where it is."

"There's **no version of this where you end up worse off**. The only thing you lose by not applying is the money you're overpaying. Fair enough?"

CALIFORNIA, 65 OR OLDER

Skip 4B.3 and go to **4B.5**.

4B.3 Umbrella pitch

"Now that's the medical side. Something I do want you to know before we continue is that even with a supplement you are exposed to **non-approved Medicare expenses**... are you familiar with that?"

"Your supplement is the best medical coverage on the market, I'll say that all day. If Medicare approves it, your plan pays it. But god forbid you have a bad health year, and have to deal with some critical illnesses or recover in a facility or the comfort of your own home — that is where the non-approved expenses start to creep in. Is that going to be a concern for you?"

Non-approved expense	Cost
Custodial care in a nursing home	\$6,000 / month
Medical professional for recovering at home	\$5,000 / month
Eliquis — after a stroke	\$500 / month
Keytruda — cancer	\$10,000 / dose
Opdivo — cancer	\$10,000 / dose

"So the supplement handles the medical bills, and then what we call **umbrella coverage** covers those non-approved expenses."

Then the Gold / Silver / Bronze write-down, exactly as in 4A.5, closing on: "...which option do you feel like you can comfortably afford on top of your current supplement plan, because we don't know if we can get you approved for that new supplement price just yet?"

4B.4 Three step close

"Based on everything you shared today, [read back your notes], my formal recommendation is the [Gold/Silver/Bronze] umbrella at **\$(X)** a month, and submitting the Medicare Supplement application with **[new carrier]** to qualify you for that lower price."

PAUSE ... the first one to talk loses.

THREE STEPS

1. We get your umbrella coverage set up for **\$(X)** a month.
2. We put in the Medicare Supplement application with **[new carrier]** to see if you qualify.
3. Your new coverage goes into effect on **[effective date]** — and that's the day we cancel the old policy, if you are approved for the new one.

"Step 1 and 2 we can knock out today. The reason we handle both today is because they go through underwriting, and I want to know where you stand."

"You do not cancel anything today. You do not cancel anything until the new plan is approved and active."

"I'll handle the entire enrollment process for you... and what's a good address to send policy packets and information to?"

4B.5 California variant — 65 or older, no ancillary

*Run 4B.1 and 4B.2 exactly as written. Do not present Gold, Silver or Bronze. **The supplement itself is the sale here.***

"So [first name], based on what you're paying now and what I'm seeing on my end, my recommendation is straightforward. Two steps, write them down."

TWO STEPS

1. We put in the application today to see if we can get you the exact same coverage for less than you're paying [carrier].
2. Your new coverage goes into effect on **[effective date]** — and that's the day, and only that day, we cancel the old policy.

"The reason we do this today instead of thinking about it is that this goes through underwriting, and **underwriting only gets harder as time goes on.** Your health today is the best it's going to be for this application. Every month you wait is a month you're overpaying, and a month of health history that can get in the way."

"And to be very clear, you do not cancel anything today. Not until the new plan is approved and active. You are never without coverage for a single day."

"Now let me get everything pulled up for us. And what was going to be a good street address for you so I make sure you receive your packages and cards?"

*Handle objections and **always end with a pre-emptive close.***

5 · Medicare Education — T65 only

How you get onto Medicare branch point

ON SOCIAL SECURITY

"So [name], what is going to happen is, since you are on Social Security, you are going to be **automatically enrolled** into Original Medicare Part A and Part B."

NOT ON SOCIAL SECURITY

"So [name], since you are not on Social Security, we will have to get you **manually enrolled** into Part A and Part B — and we will make this process as easy as possible for you. When the time comes, three months before your birth month, we will be sending you the instructions to get this done, and James has a very detailed video to help you with this step. It shouldn't take long — maybe about 10 minutes."

"Now, Original Medicare is what we call **the foundation of Medicare**. I believe James mentioned this in the webinar, but let me break it down for you again."

Introduction to Original Medicare

"**Part A** covers hospital expenses and is premium-free."

"**Part B** covers doctor visits and medical services and has a monthly premium of **\$202**."

IF ON SOCIAL SECURITY

"Since you're on Social Security, the Part B premium will be **deducted directly from your check**, so you won't need to worry about making separate payments."

IF NOT ON SOCIAL SECURITY

"Since you are not on Social Security, they are going to **bill you for the first quarter** for your Part B premium, and after that we can set up your banking info to do a monthly draft rather than paying it quarterly."

"Unfortunately, there's no way to avoid the Part B premium — it's just a standard Medicare requirement. Any questions so far?"

The exposure in Original Medicare alone

"Now, while Original Medicare is the foundation, we **never** recommend staying on it alone without additional coverage, because there's a lot of financial exposure with just Original Medicare."

"The biggest risk is the **20% coinsurance with no limit**. A \$100,000 medical bill means you're responsible for **\$20,000** out of pocket. And there is no cap on these costs, leaving you financially vulnerable."

"So to protect you, we'll discuss two options: a **Medicare Supplement plan**, also called Medigap, and **Medicare Advantage**, which is Part C. Before I go into the details, do you have any questions so far?"

"Do you have a piece of paper in front of you right now?"

Do not move on until they have pen and paper.

6 · Medicare Supplement education

"This is what we call the **Cadillac of health insurance**, because it provides the best coverage available. Now, it does come at a higher cost."

✓ COMPREHENSIVE COVERAGE

"After you meet a **\$283 annual deductible**, you are 100% covered for any medical costs. No matter the bill — \$100,000 or even a million dollars — you won't pay anything beyond that \$283 deductible. The reason it's called Medigap is because it covers the gaps that Original Medicare doesn't cover, that 20%. Does that make sense?"

Get confirmation before moving forward.

✓ NO NETWORK RESTRICTIONS

"You can see any doctor or hospital nationwide as long as they accept Medicare. So no concerns about being in or out of network."

✓ NO PRIOR AUTHORIZATION

"Medicare Supplement plans are federally regulated, meaning that by law they must cover anything Original Medicare does — so no insurance denials. If Medicare approves a procedure, your supplement plan automatically has to cover the rest."

THE GUARANTEED ISSUE WINDOW

"One thing to note before I finish up the Medicare Supplement plan is that you only have **one guaranteed issue period** to get a supplement plan, and that is when you first get onto Medicare. This means that if you elect not to go this route, if you want to switch in the future you will have to go through **medical underwriting** and can possibly be **denied** for that coverage later on."

"I know that's a lot of information, but do you have any questions on how a Medicare Supplement plan functions?"

7 · Medicare Advantage (Part C) education

"This is a more affordable option compared to the supplement plan, and it's structured differently."

✓ LOW MONTHLY COST

"Many plans have low monthly premiums, and it includes your medical, prescription drug coverage, and additional benefits like **dental, vision and hearing** at no extra cost."

△ PAY-AS-YOU-GO MODEL

"Unlike Medicare Supplement plans, you will pay co-pays and co-insurance for each service — doctor visits, hospital stays. Less predictable expenses if you have frequent medical needs."

"Think of Medicare Advantage as a **better version of your current health insurance coverage**. It essentially works the same way. I'm sure you have copays and coinsurance when you need medical care — Medicare Advantage works very similarly. The reason we say 9 times out of 10 it is a better version is because the plans usually have little to no deductible and very affordable copays."

△ NETWORK RESTRICTIONS

"You must use doctors and hospitals within your plan's network to get full coverage. You can go out of network, but expect higher costs."

"That's the general overview of Medicare Advantage — any questions on how a Medicare Advantage plan functions?"

8 • Pricing, recommendation and close

"Now that we've gone over the benefits of both plans, let's talk about cost."

Medicare Supplement (Medigap)	\$ / mo
Plan G premium	
Part D drug plan	≈ \$10
Umbrella (\$50 / \$100 / \$150)	
Total monthly — one price	

"Once you meet the small annual deductible of \$283 for the year, you owe **\$0** for covered medical services — so no surprise bills."

Medicare Advantage (Part C)	\$ / mo
MAPD premium (usually \$0)	
Umbrella (\$50 / \$100 / \$150)	
Total monthly — one price	

"You'll just have some small copays for doctor and specialist visits."

California and 65 or older: the Supplement umbrella line is dental only (\$50, or nothing if dental does not matter to them). Drop the umbrella line from the Advantage table entirely.

Which option are they leaning toward?

"Now that we've gone through both options and the detailed pricing breakdown, which route do you feel yourself leaning toward as of now?"

PAUSE. *Let them answer.*

"Okay, so you're leaning toward [Option 1 / 2]?"

Get their confirmation again.

Formal recommendation

"Based on everything you shared today — *(read back your notes from the call)* — my formal recommendation would be the **[Supplement / Advantage] plan**, with umbrella coverage at **\$(X)** a month, for about **\$(total)** a month all in."

PAUSE. The first one to talk loses.

The 3 simple steps branch point

"Do you still have that piece of paper? Perfect — I want you to write this down, because this is how we will get everything done, in these 3 simple steps."

SCENARIO A — T65, NOT ON SOCIAL SECURITY

1. We get your umbrella coverage set up for **\$(X)** a month.
2. We manually enroll you into Original Medicare Parts A & B three months before your birth month — it takes about 10 minutes.
3. We enroll you into your **[Supplement — 6 months / Advantage — 3 months]** before your 65th birthday.

SCENARIO B — T65, ON SOCIAL SECURITY

1. We get your umbrella coverage set up for **\$(X)** a month.
2. Since you're on Social Security you'll be automatically enrolled into Parts A & B — we just confirm that's all set.
3. We enroll you into your **[Supplement — 6 months / Advantage — 3 months]** before your 65th birthday.

Clients already on Medicare use the 3 step close in section 4 instead.

California and 65 or older with no dental sale: step 1 is removed and this becomes a two-step close. If dental matters to them, step 1 is the \$50 dental umbrella on the Supplement side.

"Step 1 is the only thing we need to knock out today."

Why we do this today

"The reason we want to go ahead and handle this now is because we need to make sure we can get you **approved through underwriting**. That way, if for some reason one company doesn't approve of you, we still have time to pivot and find another option before your effective date."

"The other important part is once you're approved, this **locks in your rate based on your age right now** — so it's never going to jump because of your health later on."

"I'll handle the entire enrollment process for you. It's only a few quick health questions and we should be able to get everything wrapped up in the next few minutes. Now let me get everything pulled up for us."

"And what was going to be a good street address for you, so I make sure you receive your packages and cards?"

*Move straight into the application. Don't ask permission — assume it. Then handle objections and **always end with a pre-emptive close**.*

Mohr Insurance Services · Dynamic sales script reference, script version 5. The interactive version routes the avatar and prices the umbrella for you — this document is the offline backup.